NCCP 00259 · VERSION 6

Vinorelbine capsules · advanced breast cancer

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4,80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity.60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Low blood counts, infection and bleeding

☐ Nausea, vomiting and fatigue

☐ Constipation or bowel obstruction

☐ Nerve effects and uncommon lung/heart toxicity

☐ Incorrect weekly dosing or damaged capsules

Purpose and alternatives for the person’s advanced breast cancer, including other systemic treatment or supportive care according to goals. Possible benefit is uncertain; response is reviewed and treatment can be paused, changed or stopped. Low blood counts, infection/sepsis, fatigue, nausea/vomiting, bowel problems, neuropathy, hair change and uncommon serious lung/heart/allergic toxicity. Fertility and pregnancy planning before treatment; discuss sperm/egg preservation where relevant. Do not breastfeed. HPRA supplements: contraception during treatment and for 7 months afterwards for women,4 months for men; confirm the exact product and individual advice.

Individual benefit, alternatives and concerns

Agreed checks and next review

Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.20 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending